

COOK COUNTY MEDICAL CENTER

Department of Emergency Medicine
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EMERGENCY DEPARTMENT NOTE

Patient: Marcus D. Whitfield **MRN:** PED-2025-04471
DOB: 03/03/2016 **Age:** 9 years, 10 months **Sex:** Male
Date/Time: 01/22/2026 10:42 AM **Mode of Arrival:** Walk-in with caregiver
Chief Complaint: Breathing difficulty, wheezing
Triage Acuity: ESI Level 3

SUBJECTIVE

Marcus is a 9-year-old male brought in by his foster parent for wheezing and breathing difficulty that started overnight. The caregiver reports that Marcus was coughing throughout the night and woke up with audible wheeze this morning. She administered albuterol via nebulizer at home with partial improvement. He has a history of severe persistent asthma with three prior intubations (2022, 2023, 2024). No fever, no URI symptoms, no vomiting. Caregiver states he has been snoring loudly every night and that she has witnessed pauses in his breathing during sleep.

History obtained from foster parent. Marcus is minimally verbal during the encounter.

MEDICATIONS (PER CAREGIVER)

- Symbicort 80/4.5 mcg 2 puffs BID
- Albuterol MDI PRN
- Risperidone 1.5 mg QHS
- Methylphenidate ER 27 mg QAM
- Clonidine 0.1 mg QHS
- Montelukast 5 mg QHS
- Ferrous sulfate 325 mg daily
- Melatonin 3 mg QHS

OBJECTIVE

Vital Signs: Temperature 98.6 F, Heart Rate 104 bpm, Respiratory Rate 24, Blood Pressure 88/54 mmHg, SpO2 89% on room air (improved to 91% on 2L NC then 95% on 4L NC). Weight: 25.9 kg.

General: Alert, anxious-appearing male in mild respiratory distress. Thin habitus. **HEENT:** Oropharynx notable for tonsillar hypertrophy, tonsils graded 3+ bilaterally. No pharyngeal erythema or exudate. Multiple cafe au lait macules noted on neck and trunk. **Lungs:** Bilateral expiratory wheeze, no rales, no rhonchi. Moderate air movement. Accessory muscle use present but not severe. **Cardiovascular:** Regular rate and rhythm, no murmur. **Abdomen:** Soft, non-tender, non-distended. **Skin:** No cyanosis. Cafe au lait macules as noted. **Neuro:** Alert, limited interaction, intermittent hand-flapping (per caregiver, baseline behavior).

LABS ORDERED

TSH and Free T4 (to evaluate for thyroid-related contribution to vital sign abnormalities). Urine drug screen (standard per ED protocol for altered vital signs in pediatric patient). Blood lead level (history of pica, elevated lead on prior testing). Results: TSH within normal limits, UDS negative, lead 18 mcg/dL.

ED COURSE

Albuterol nebulizer x 2 administered in the ED with improvement in wheeze and work of breathing. SpO2 improved from 89% to 95% on supplemental oxygen. After second nebulizer treatment, SpO2 maintained at 94-95% on 2L NC. Patient appeared comfortable. Wheeze diminished but not fully resolved. Given history of 3 prior intubations, patient was monitored in the ED for 3 hours. SpO2 stabilized at 95-96% on room air by end of observation period.

ASSESSMENT

1. Acute asthma exacerbation, mild-to-moderate, responsive to bronchodilator therapy. Given history of severe persistent asthma with multiple intubations, this patient warrants close outpatient follow-up.

2. Hypotension (BP 88/54) — likely medication-related in the setting of clonidine and risperidone use. No evidence of acute hemodynamic compromise. Recommend outpatient evaluation.

3. Tonsils 3+ — incidental finding. May warrant ENT evaluation if symptomatic.

PLAN / DISPOSITION

- Discharge to home with foster parent.
- Continue current asthma controller regimen (Symbicort 80/4.5 mcg BID, albuterol PRN).
- Albuterol nebulizer treatments q4h PRN for 48 hours, then PRN.
- PCP follow-up within 3 to 5 days for asthma reassessment and blood pressure recheck.
- Return precautions reviewed with caregiver: return immediately if increased work of breathing, blue lips, inability to speak, or SpO2 < 92% on home pulse oximeter.
- Echocardiogram recommended as outpatient to evaluate for structural contribution to hypotension.

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Electronically signed: 01/22/2026 at 15:08 CST
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